Ministry of Health Sri Lanka

World Health Organization

Meeting Report

Regional Commemoration of World Diabetes Day 2024

21–22 November 2024
Colombo, Sri Lanka

WHO Global Diabetes Compact

SEA-NCD-114

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1 Background

Globally, around 828 million people have diabetes, and two million deaths are attributed to diabetes every year. In the WHO South-East Asia (SE Asia) Region, more than 246 million adults 30 years and above are estimated to have diabetes[1]. Diabetes and its complications (blindness, kidney failure, heart attack, stroke, and lower limb amputation) bring about substantial economic loss to people with diabetes and their families, and to health systems and national economies. More than 50% of people with diabetes are unaware of their status and 177 million people living with diabetes in the region. are not on treatment

In April 2021, the World Health Organization (WHO) launched the Global Diabetes Compact (GDC) with the vision of reducing the risk of diabetes and ensuring that all people diagnosed with diabetes have access to quality care and treatment that is equitable, comprehensive, and affordable. The Compact guides actions through six strategic workstreams, including prevention and diagnosis, quality care, monitoring and surveillance, medicines and technologies, technical guidance and innovation, and governance and partnerships. The Seventy-fifth World Health Assembly adopted the first-ever five global coverage targets for diabetes for 2030 -WHA75 (11)[2] and recommended to strengthen and monitor diabetes responses within national noncommunicable disease (NCD) programmes .

The Seventy-fifth session of the WHO Regional Committee for South-East Asia endorsed the Implementation Roadmap for the prevention and control of noncommunicable diseases in South-East Asia 2022–2030, along with a resolution (SEA/RC75/R2)[3], urging the countries to adapt and implement the roadmap, in accordance with their national priorities and contexts. The roadmap is aligned with the WHO NCD targets for 2030 and five global diabetes coverage targets. The resolution of Seventy-sixth WHO Regional Committee 'SEAHEARTS: Accelerating Prevention and Control of Cardiovascular Diseases in the South-East Asia Region (SEA/RC7/R5)' endorsed four milestones including milestone to place 100 million people with diabetes and hypertension on protocol-based management by 2025 as a step to improve diabetes coverage and control in primary health care.

There is clear evidence that the countries have made steady progress in reaching SEAHEARTS milestones. The policy and programmatic initiatives taken by countries to address tobacco control, obesity, trans-fatty acid elimination, and physical inactivity are being identified as good practices globally. A total of 23 million people with diabetes in

1 Worldwide trends in diabetes prevalence and treatment from 1990 to 2022: a pooled analysis of 1108 population-representative studies with 141 million participants - The Lancet

2 Resolutions and decisions of Seventy-fifth World Health Assembly - WHA75 (11), page 47 https://apps.who.int/gb/ebwha/pdf_files/WHA75-REC1/A75_REC1_Interactive_en.pdf#page=1

3 https://www.who.int/southeastasia/publications/i/item/SEA-RC76-R5

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the region have been placed on protocol-based management by mid-2024 along with another 37 million with hypertension have been placed on protocol-based management indicating the potential of the region to reach the target of 100 million by December 2025[4]. Despite these efforts, countries in the Region continue to face challenges with high rates of undetected, untreated, and uncontrolled diabetes.

World Diabetes Day "Breaking Barriers, Bridging Gaps" commemorated on the 21-22 November served as a reminder to the need to collectively work to overcome the growing burden of diabetes globally and in WHO South-East Asia Region. The theme emphasized the importance of tackling the challenges faced by individuals, communities, and health systems to access diabetes prevention and management.

In this context, the Ministry of Health, Government of Sri Lanka, in collaboration with WHO, conducted an advocacy event titled *Regional Commemoration of World Diabetes Day 2024-'Breaking barriers, Bridging gaps'* to advocate for the integration of the Global Diabetes Compact's Key Asks into national health agendas across countries and frame the 'Colombo Call to Action: strengthening diabetes prevention and control in the WHO South-East Asia Region'.

Specific objectives

- Advocate for the implementation of priority actions to achieve the global diabetes coverage targets by 2030.
- Facilitate cross-learning by sharing good practices and leveraging experiences from both within and beyond the region.
- Convene diverse stakeholders across sectors to discuss successes, challenges, and future needs related to the Key Asks of the GDC in the regional context.
- Identify opportunities, synergies, partnerships and priority actions to strengthen national responses to the growing diabetes burden in the region.

4 Regional consultation on SEAHEARTS Monitoring Framework and measuring progress on milestones https://www.who.int/southeastasia/publications-detail/SEA-NCD-113

2 Summary of the proceedings

Opening session

The opening session commenced with Dr S C Wickramasinghe, Deputy Director General (NCD), Ministry of Health, Sri Lanka delivering the welcome remarks. She highlighted the public health importance of addressing the burden of diabetes in terms of global, regional and country context. She emphasized the significance of World Diabetes Day in the context of Sri Lanka's efforts to place 1 million people with hypertension and diabetes on the protocol-based management by 2025 to contribute to SEAHEARTS milestone. She also briefed the participants on the objectives and rationale of two days meeting along with expected outcomes.

Dr Tedros Adhanom Ghebreyesus, WHO Director-General conveyed his remarks through video message stating the high burden of diabetes in terms of the number of people living with diabetes having quadrupled over the past three decades with the number surpassing 800 million globally. He requested countries, partners and stakeholders to intensify efforts to meet global targets and ensure better prevention, diagnosis, and management of diabetes.

Inaugural ceremony of the Regional Commemoration of World Diabetes Day

Ms Saima Wazed, Regional Director, WHO South-East Asia in her video message acknowledged Sri Lanka's leadership in addressing diabetes and promoting healthier lifestyles. She commended South-East Asia's commitment to SEAHEARTS milestones and emphasized urgent need for collective action in diabetes prevention and management and urged governments and partners to act decisively to meet the GDC targets by 2030 (annex 1).

Dr Guy Fones, Director (NCD) a.i., WHO Headquarter emphasized on the commitment by the member States to achieve five global diabetes targets by 2030 and acknowledged the initiative of the SE Asia region on SEAHEARTS milestones. He highlighted that the proposed outcomes of 'Call to Action' need to be bold, actionable and measurable to address the root causes of the burden of diabetes in the SE Asia region. He stated that the Call to Action should also potentially inform the needs of the region to the fourth UN High Level meeting on NCDs.

Ms Kiara Powar represented people living with diabetes in a video message and highlighted the importance of addressing stigma and misconception about diabetes in the society. She urged the healthcare sectors to provide affordable accessible and empathetic care. Society and families were requested for the support and diabetic community to stay strong and motivated and those living with diabetes were encouraged to share experiences for shaping policies and programmes.

The SEAHEARTS Monitoring Platform developed as a digital dashboard designed to track progress on four regional SEAHEARTS milestones was also launched as part of the opening session of the event. WHO developed this digital tool to fulfill its mandate to provide a platform to update the progress of the SEAHEARTS milestones in the region. The platform also features case studies, good practices, key country interventions and learning resources for prevention and control of cardiovascular diseases and diabetes in the region, thus enabling knowledge sharing, documenting learning and promoting collaboration among countries.

Dr P G Mahipala, Secretary, Ministry of Health, Sri Lanka in his remarks welcomed the participants and acknowledged for WHO's leadership in collaborating with Sri Lanka for the event. He reaffirmed Sri Lanka's commitment to the key objectives of the GDC and indicated health sector reforms, particularly in strengthening primary health care as a response to growing challenge of diabetes. Dr Mahipala indicated that the proposed Call to Action will serve as a shared framework in identifying effective solutions and strengthen cross-border collaboration in addressing limited awareness, unequal access to care, and social stigma due to diabetes. He reiterated that by working together, nations can break barriers to healthcare access, ensuring a healthier future for individuals living with diabetes.

Dr Samiddhi Samarakoon, Director (NCD), Ministry of Health, Sri Lanka delivered the vote of thanks.

Technical session A: Key priorities of the Global Diabetes Compact

Session moderators Dr S.C. Wickramasinghe, Deputy Director General (NCD), Ministry of Health, Sri Lanka Dr Guy Fones, Director (NCD) a.i., NCD Department, WHO Headquarter	
Essentials of the WHO Global Diabetes Compact: what it means at regional level*(virtual)*	Dr Bianca Hemmingsen Medical Officer (Diabetes Control), WHO HQ
Inspiring Success: Global lessons from the key asks of WHO Global Diabetes Compact	Dr Nikhil Tandon Chair, WHO Global Technical Advisory Group on Diabetes
Synthesis of country efforts and an overview of the SEAHEARTS initiative to accelerate the prevention and control of diabetes in WHO South-East Asia Region	Dr Pradeep Joshi Technical Officer (NCD), WHO South-East Asia Regional Office

Session objectives:

- To provide an outline on the Key asks of Global Diabetes Compact and an overview of the SEAHEARTS initiative regional guidance onto accelerating the prevention and control of diabetes in WHO South-East Asia Region
- To present a synthesis of country efforts/learnings of implementing GDC

Setting the context for technical sessions

Dr Hemmingsen presented that GDC was launched in April 2021 and is supported by governments, health ministers, and non-state actors, with a primary goal of improving access to diabetes treatment, prevention, tracking, education, funding, innovation, and partnerships and was adoption by WHO Member States with a resolution on diabetes at the World Health Assembly in May 2021 (WHA75 (11)[5]. She presented the set of global coverage targets for diabetes care (Box1) and the eight components of the WHO Global Diabetes Compact Key Asks (Box 2).

Key workstreams of the GDC were explained as access to essential diabetes medicines and associated health technologies; technical product development; prevention, health promotion and health literacy; country support; research and innovation; and governance.

Box 1. The five global diabetes coverage targets to be reached by 2030:

- 80% of people with diabetes are diagnosed;
- 80% of people with diagnosed diabetes have good control of glycaemia;
- 80% of people with diagnosed diabetes have good control of blood pressure;
- 60% of people with diabetes of 40 years or older receive statins; and
- 100% of people with type 1 diabetes have access to affordable insulin and blood glucose self-monitoring.

Citing key examples of specific technical work she highlighted expanding access to essential medicines and technologies including improving availability of insulin, glucose meters, and HbA1c devices, with WHO prequalifying human insulin and glargine, technical support to implement WHO's PEN interventions and HEARTS-D package for diabetes management in primary healthcare settings, provision of emergency NCD kits and developing new global guidelines on diabetes monitoring. She also highlighted the research and innovation efforts which include the Global Diabetes Research Agenda (WHO-PAHO collaboration) and AI-based technology standards for early diabetic retinopathy detection. The need to support countries for strengthening of policy, funding, technology, and education were cited as essential to achieving global diabetes targets by 2030.

Highlighting the Global lessons from the key asks of WHO Global Diabetes Compact, Dr Nikhil Tandon presented the following country examples.

- Indonesia – local insulin production to reduce dependency on imports.
- Thailand – Universal Health Coverage (UHC) includes comprehensive diabetes prevention and treatment, increasing diagnosis and treatment rates.
- India – I-TREC initiative integrates technology and task-sharing to enhance diabetes and hypertension management
- Singapore – adopting a nationwide approach with prevention programs, health promotion campaigns, and public health engagement.

5 Page 47- A75_REC1 (https://apps.who.int/gb/ebwha/pdf_files/WHA75-REC1/A75_REC1_Interactive_en.pdf#page=1)

Box 2. The eight components of the WHO Global Diabetes Compact (Key Asks):

1. Collaboratively unite stakeholders, including people living with diabetes, around a common agenda (UNITE)
2. Integrate diabetes prevention and management in primary health care and universal health coverage (INTEGRATE)
3. Close research and normative gaps while spurring innovation (INNOVATE)
4. Improve access to diabetes diagnostics, medicines, and health products—particularly insulin—in low-income and middle-income countries (TREAT)
5. Develop global coverage targets, accompanied by a global price tag that quantifies the costs and benefits of closing the gap between people who can access diabetes services and those who cannot (TRACK)
6. Improve diabetes care for people in humanitarian emergencies (FUND)
7. Improve health literacy and understanding of diabetes prevention and management (EDUCATE)
8. Build back better, learning from experiences of the COVID-19 pandemic (POWER BACK)

Dr Pradeep Joshi presented an overview of the SEAHEARTS initiative to accelerate the prevention and control of diabetes in WHO South-East Asia Region He described that the region has embarked on an innovative approach of SEAHEARTS initiative to reduce the cardiovascular disease burden bringing together efforts to control tobacco, reduce salt consumption and eliminate trans-fatty acids, along with improved coverage of hypertension and diabetes services. aiming to achieve a set of collective milestones by 2025. Reporting the progress as of June 2024, he highlighted that 59 million people with hypertension and diabetes are placed on protocol-based treatment, while 2.1 billion people are covered WHO SHAKE measures, and 2.04 billion people are protected from trans-fatty acids.

He also summarized the following as country-specific efforts showcased through poster presentation.

- Bangladesh – mobilizing communities through religious leaders and culturally adapted materials,
- Bhutan – integrating traditional health practices with modern care and set district-wise targets to reduce care gaps
- India – demonstrating strong political commitment with its 75 by 25 initiative, leveraging Health and Wellness Centers for telehealth and diagnostics.
- Indonesia – integrating TB-DM management in primary healthcare and used HbA1C testing for treatment monitoring. Maldives reoriented its health system, focusing on capacity building and population tracking

- Nepal – implementing opportunistic screening and PEN Plus interventions for Type 1 diabetes
- Thailand – promoting pro-health taxes and implementing diabetes remission in NCD clinic with quality assessment,
- Malaysia – introducing structured lifestyle medicine programs in primary healthcare.

Interaction during poster presentation session

Following were key takeaways from this session.

- WHO's Global Diabetes Compact has the vision to reduce the risk of diabetes and ensure that all people who are diagnosed with diabetes have access to equitable, comprehensive, affordable and quality treatment and care. There are seven Key Asks: UNITE, INTEGRATE, INNOVATE, TREAT, TRACK, FUND and EDUCATE.
- WHO Global Diabetes Compact operates through six key workstreams to support countries in strengthening the national diabetes response.
- Several WHO Governing Bodies Resolutions and Decisions exists that can be leveraged to strengthen diabetes prevention and management and achieve the five global diabetes coverage targets by 2030.
- Significant unmet need in WHO South-East Asia Region with every one in four adults living with diabetes and only one in three adults with diabetes receiving treatment. Diabetes prevalence among women has surpassed that of men.
- There is a need to better align regional and national initiatives with the Global Diabetes Compact to drive progress in diabetes prevention and management.
- The progress made on the SEAHEARTS initiative in the WHO South-East Asia Region acts as a strong foundation to strengthen diabetes prevention and management.

Technical session B: Bridging access – closing the gap for diabetes essential medicines, technologies and health products

Session moderators Mr Nayaz Ahmed, Deputy Director General, Ministry of Health, Maldives Dr Nalika Gunawardena, Regional Adviser (NCD), WHO South-East Asia Regional Office	
Barriers to available, affordable and quality-assured insulin and key pathways to access	Dr Margaret Ewen, Senior Projects Manager, Health Action International
Bringing diagnostic closer: improving access to diabetes technologies in primary health care	Dr Beatrice Vetter, Director NCDs, FIND

Moderated discussion with panelists on good practices and solutions based on the country level experience to improve availability and access to diabetes medicines, and technologies

Panelists:

- Dr Esti Widiastuti Mangunadikususmo, Senior Health Epidemiologist, Ministry of Health, Indonesia
- Dr Anupam Prakash, Head of Medicine Department, Lady Hardinge Medical College, India
- Dr Helder Juvinal Neto da Silva, PEN Unit Officer, Ministry of Health, Timor Leste
- Ms Kim Uhjin, Regional Adviser (Essential drugs and medicines), WHO South-East Asia Regional Office

Exploring availability and access to essential medicines, technologies and health products

Session objectives:

- To discuss the key challenges related to improving the availability of diabetes medicines, technologies and heath products and global efforts to overcome the challenges
- To share the good practices and solutions based on the country level experience to improve availability and access to diabetes medicines, and technologies
- To identify the catalytic actions by the countries to improve availability and access to diabetes diagnostics, medicines, and technologies in the WHO South-East Asia Region

Dr Margaret Ewen's context setting presentation focused on the barriers to accessing affordable, quality-assured insulin and the pathways to improve access. She highlighted that optimal diabetes management requires not only insulin or oral tablets but also delivery devices, blood glucose monitoring tools, HbA1c testing, and healthcare provider support.

She indicated several challenges in the insulin supply chain, including manufacturing, product registration, selection, pricing, reimbursement, procurement, supply, prescribing, dispensing, and use. In summary

- Biosimilar manufacturers face underutilized capacity and regulatory complexities, limiting competition and driving up prices.
- Insulin prices are often unaffordable, especially for low-wage earners, and the cost of blood glucose monitoring tools adds to the financial burden.
- Procurement and supply challenges include inadequate need assessment, inefficient government procurement, lack of transparency, and significant mark-ups and taxes that inflate prices.
- Dispensing issues arise when insulin is only available at higher-level health facilities, particularly outside urban areas.
- Prescribing and use are hindered by outdated clinical guidelines, lack of healthcare professional training, and limited understanding of biosimilars. Patients often face inadequate training, stigma, and poor access to monitoring tools.
- UHC for diabetes programs is often absent or fragmented, and global and national market inequities and inefficiencies further hinder access.

To address these challenges, Dr Ewen proposed several actions as follows

- National health insurance or UHC should cover all people with diabetes, including a standard care package with insulin, delivery devices, meters, test strips, and HbA1c testing, free of financial hardship.
- Governments should accurately assess needs, ensure adequate financing, and adopt good procurement practices. Transparency in prices and volumes is crucial, and supply chain mark-ups and taxes should be regulated.
- The WHO should include biosimilars in its prequalification program and support national regulatory authorities.

- Education for healthcare providers and patients, along with public awareness campaigns, is essential. Regular monitoring of availability, prices, and rationing can help identify and address issues.

Dr Beatrice Vetter's presentation focused on improving access to diabetes diagnostic devices and technologies, particularly in primary healthcare settings. She highlighted the significant diagnostic gap in diabetes, with less than 50% of people diagnosed and fewer than 1 in 4 successfully treated. Closing this gap could prevent an estimated 0.6 million deaths and avert 14.7 million disability-adjusted life years (DALYs) annually.

The presentation brought the importance of point-of-care (POC) testing devices, such as HbA1c and blood glucose meters, in improving diagnostic rates. These devices, which can be used in primary care, community settings, and for self-testing, offer quick results and ease of use, making them valuable tools for early diagnosis and management of diabetes. However, she indicated that while many POC devices are available, their accuracy often falls short of manufacturers' claims, underscoring the need for careful selection and independent validation.

The presentation outlined key facilitators and barriers to implementing POC testing.

- Facilitators include quick turnaround times, ease of sample collection, and improved service delivery.
- Barriers include implementation costs, lack of regulatory and accreditation mechanisms, and the need for training.

Importance of validating POC devices in real-world settings, particularly in low- and middle-income countries (LMICs) was highlighted. Trough two case studies as below:

- Findings from a study in Peru where the availability of POC devices doubled the rate of new diabetes diagnoses compared to settings without such devices, demonstrating their potential to reduce lost-to-follow-up cases.
- independent performance evaluations in Cambodia and Nigeria, which highlighted the need for devices to be accurate under local conditions and with diverse populations. These evaluations are crucial for ensuring that POC devices perform reliably in the intended use settings and with varying levels of user training.

Dr Vetter called for the adoption of WHO-recommended POC devices at lower levels of care and emphasized the need for careful implementation to maximize their impact on diabetes diagnosis and management.

Moderated panel discussion included panelists from Indonesia, India, and Timor-Leste discussing their country-specific initiatives, while expert from WHO SEARO provided a broader perspective. The key points highlighted during moderated discussion were:

(1) Dr Esti Widistuti Mangunadikusumo from Indonesia highlighted the country's comprehensive diabetes management frameworks, which include national guidelines, care protocols, and a national formulary covering all disease conditions, including

diabetes, across all levels of the healthcare system. These frameworks ensure the utilization of analogue insulin in both public and private healthcare sectors.

(2) Discussing the India's advancements in producing biosimilars and glucometers, which are widely used and available at all health facilities, Dr Anupam Prakash highlighted that country employs an integrated drug distribution system and has introduced a digital forecasting tool to prevent drug stock-outs. However, challenges remain in accurately forecasting the needs of newly diagnosed diabetes cases, which require frequent updates to avoid shortages.

(3) Dr Helder Juvinal Not da Silva from Timor-Leste shared the country's progress in addressing insulin shortages. Until 2024, Timor-Leste had no insulin, but it has since started importing insulin from Indonesia and India, including short-acting, mixtard, and long-acting analogues. The country has also established a national regulatory directorate to ensure the quality of imported medicines, including insulin. The primary challenge lies in selecting reliable suppliers.

(4) Important aspect of high expenditure on analogue insulin in the South-East Asia region, and exclusion of insulin and fixed drug combinations for diabetes from national essential medicines lists was indicated by Ms Kim Uhjin from WHO SEARO. She brought the point that this leads to irrational drug use, as private practitioners frequently prescribe newer, more expensive drugs outside the essential list. Weak public procurement processes further exacerbate the issue. Ms Uhjin proposed four key interventions: strengthening public procurement, regulating medical prices, promoting rational drug use, and increasing public spending.

Following were **Key takeaways** from this session.

- strengthen the capacity of national regulatory authorities to ensure all diabetes products on the market are quality-assured (including the evaluation of biosimilar insulins).
- adopt robust forecasting tools for realistic quantification of requirements as per treatment protocol (including up-to-date patient registries).
- strengthening national procurement mechanisms by incorporating market assessments, supplier prequalification, centralized tendering and other best practices for goods procurement.
- strengthen distribution systems, including the use of digitalized systems and monitoring stock-outs.
- scale up the cold chain infrastructure to ensure the quality of insulin.
- regulate prices and distribution mark-ups, and eliminating taxes on essential diabetes medicines, blood glucose monitoring devices and testing strips.
- monitor availability and prices in facilities.
- in addition to testing blood glucose using a meter and test strips, prioritize access to HbA1c testing at primary health care facilities.

Technical session C: Universal diabetes care – Integrating care and tracking progress

Session Moderators

Mr Saroj Kumar, Director, Ministry of Health and Family Welfare, India

Dr Gampo Dorji, Team Lead (NCD and Health systems), WHO Nepal

Integrating diabetes prevention and management at primary health care and universal health coverage: examples from low- and middle-income countries	Dr Hans V Hogerzeil Professor, Faculty of Medical Science, University of Groningen

Moderated discussion with panelists on country level experience in integrating diabetes prevention and management in PHC for UHC, and in tracking coverage targets for diabetes care

Panelists:

- Dr Churit Tengtrisorn, Medical Officer, Ministry of Public Health, Thailand
- Dr Karchung, Deputy Chief Program Officer, Ministry of Health, Bhutan
- Mr Abdulla Muaz Adam, Public Health Coordinator, Ministry of Health, Maldives
- Dr James Duah, Deputy Executive Director, Christian Health Association of Ghana

Session objectives:

- To provide an overview of the key challenges to integrate diabetes management in primary health care and successful approaches/ practices to overcome the challenges
- To share the good practices and solutions based on country level experience in integrating diabetes prevention and management in PHC for UHC, and in tracking coverage targets for diabetes care
- To identify the catalytic actions by the countries to integrate diabetes management in primary health care and to track the progress towards achievement of global diabetes targets in the WHO South-East Asia Region

The context setting presentation by Dr Hans V. Hogerzeil's focused on integrating diabetes prevention and management into primary healthcare (PHC) and UHC systems, drawing examples from LMICs. Dr Hogerzeil pointed out the importance of integrating diabetes care within PHC systems, ensuring access to essential medicines like insulin, and strengthening referral services through examples. The presentation highlighted case studies to emphasize the integrated management:

- Insulin donation programs in 43 LMICs, which demonstrated that type 1 diabetes can be successfully managed at the district hospital level, improving health outcomes and strengthening health systems.

Discussion on country-level experiences in integrating diabetes management into primary health care

- In Peru, pooled procurement of human insulin improved availability at primary care facilities, but affordability remains an issue in the private sector.
- Tanzania's transition from a donor-supported insulin program to a national diabetes program through establishing dedicated diabetes clinics, updating clinical guidelines, integrating diabetes care into national health insurance, and training healthcare workers.
- In Uganda and Nepal, where insulin is often unavailable at primary healthcare centers, forcing patients to rely on private pharmacies with higher prices.
- He also pointed out the lack of international solidarity for NCDs like diabetes, compared to HIV/AIDS.

In summary, Dr Hogerzeil's presentation underscored the need for systemic changes to ensure affordable, accessible diabetes care in LMICs, with a focus on equity, efficiency, and strong government leadership.

Moderated panel discussion featured country-specific approaches to tackling diabetes from Thailand, Bhutan, Maldives, and Ghana, the salient points shared were

1. Dr Churit Tengtrisorn from Thailand emphasized the development of a comprehensive National NCD Plan, implemented at both national and sub-regional levels. The country focuses on prevention, care, treatment, and continuous management, with strong primary care supported by community health volunteers and sub-district health promotion hospitals. Thailand also leverages UHC, which covers 99.5% of the population, and various health financing streams, including insurance for labor groups, government workers, and private insurers.

2. Pioneering efforts in setting national and subnational NCD targets in Bhutan, known as the 80-80-80 target, aiming to screen, treat, and control diabetes in 80% of the population was highlighted by Dr Karchung. He highlighted the process of target setting and the plan to achieve it within five years Dr Karchung also indicated government has integrated these targets into its 13-year plan, ensuring strong governance and mainstreaming NCD targets across all health workforce levels.
3. Mr Abdulla Muaz Adam from Maldives showcased its successful pilot of an integrated PHC model, which includes a decentralized approach to health service delivery. The country is ready to scale up its NCD project nationally, supported by systematic approaches such as baseline assessments, capacity building, and digital technologies
4. Ghana's approach for leveraging existing programs like immunization and maternal and child health to reach diabetes targets was pointed out by Dr James Duah from Christian Health Association of Ghana (CHAG). He also highlighted how CHAG plays a key role in extending health services to rural areas and integrating NCD programs through innovative approaches, including mobile applications for patient connectivity and teleconsultations in rural areas.

Following were **Key takeaways** from this session.

- identify opportunities to ensure that diabetes prevention and management interventions are fully integrated into existing programmes at the primary health care level for other NCDs (e.g., hypertension), maternal and child health care, tuberculosis, and HIV services.
- integrate an essential package of diabetes diagnosis, treatment and care into universal health coverage benefit packages to ensure the delivery of services that are promotive, preventive, rehabilitative and palliative and focused on effective management throughout the life course.
- offer testing to identify individuals with diabetes, ensuring linkage to the care cascade across all levels of care, including diagnosis, treatment and management, and addressing complications.
- achieve better glycaemic control for people living with diabetes receiving care, through full implementation of nationally or sub-nationally approved treatment protocols and approaches of WHO HEARTS-D technical packages.
- provide healthy lifestyle counseling and patient education to improve prevention and management of diabetes.
- implement health management information systems, preferably electronic, to enable longitudinal tracking of people living with diabetes in both public and private sectors. These systems should generate information on national and global targets and serve as a management tool to improve care at facility level.
- invest in implementation research to foster evidence-informed diabetes prevention and treatment programme design and implementation.

Technical session D: Healthier communities, healthier living, healthier future: preventing diabetes and promoting wellbeing

Session moderators

Dr Sivarajan Ramasamy, Senior Principal Assistant Director, Ministry of Health, Malaysia

Dr Bhupinder Kaur Aulakh, WHO Representative to Bhutan

From prevention to remission: tackling diabetes with effective strategies and solutions	Dr Weerachai Srivanichakorn Associate Professor, Faculty of Medicine Siriraj Hospital Mahidol University, Thailand
Balancing blood sugar: the power of dietary interventions	Dr Khadar Vali Dudekula Independent Scientist, Vishwa Manavata Samastha, India

Moderated discussion with panelists on good practices and innovative solutions based on country level experience preventing diabetes and promoting wellbeing of those living with diabetes

Panelists:

- Dr Rahat Iqbal Chowdhury, Deputy Program Manager, Ministry of Health and Family Welfare, Bangladesh
- Dr Pushpa Raj Poudel, Senior Consultant, Ministry of Health and Population, Nepal
- Ms. N.D.C. Ruwanthi, Public Health Nursing Officer, Ministry of Health, Sri Lanka
- Dr Indah Suci Widyahening, Professor, Faculty of Medicine Universitas Indonesia

Interaction with country representatives and experts on preventing diabetes and promoting wellbeing

The presentation also explored the concept of diabetes remission as a powerful motivator for both patients and healthcare providers. Dr Srivanichakorn outlined two perspectives on diabetes remission in Thailand:

- as a strategy for all diabetes and prediabetes patients to raise awareness and promote lifestyle changes, and
- as a targeted approach for individuals with early-stage diabetes who are likely to achieve remission.

Dr Srivanichakorn highlighted the importance of intensive lifestyle interventions, such as the "2:1:1 meal box" (a balanced diet of vegetables, protein, and carbohydrates), which aligns with traditional Thai eating patterns.

ELIMINATE-Diabetes project, a cluster-randomized control study evaluating the effectiveness of intensive lifestyle interventions for diabetes remission in early type 2 diabetes was shared by him which includes economic evaluations and implementation models to assess the cost-effectiveness and scalability of these interventions in Thailand's public health system. Additionally, he discussed the development of a digital ecosystem, including mobile applications, dashboards, and chatbots, to enhance diabetes remission services and improve patient outcomes. In conclusion, Dr Srivanichakorn emphasized the need for personalized, culturally tailored interventions to address the growing diabetes epidemic.

The talk from Dr Khadar Vali talk focused on power of dietary interventions based on his extensive research on the role of diet in preventing and managing diabetes. He emphasized the significance of traditional, organic, and unpolished millets as a natural and effective solution for blood sugar regulation. His approach centers around, a group of five positive millets—Foxtail Millet, which improves insulin sensitivity; Barnyard Millet, which regulates blood sugar spikes; Kodo Millet; Little Millet, and Browntop Millet

He stressed that strategies to discourage consuming refined foods such as white rice, wheat, processed sugars, and artificial additives, as they contribute to insulin resistance and metabolic disorders. He highlighted by integrating diet, lifestyle modifications, and natural healing, he emphasized on holistic approach promotes sustainable health. His recommendations also include the consumption of seasonal, pesticide-free fruits and vegetables to enhance metabolic functions and the intake of herbal decoctions to aid in detoxification.

In conclusion, he highlighted the power of nature and traditional foods in maintaining balanced blood sugar levels and preventing lifestyle diseases. His concluding message was solution is not in synthetic interventions alone but in returning to nature, reviving traditional foods, and making conscious choices that nourish both body and mind.

Moderated panel discussion include representatives from Bangladesh, Nepal, and Sri Lanka, who shared their country-specific initiatives and lessons learned, while experts from academic institute provided broader perspective.

1. Dr Rahat Iqbal Chowdhury from Bangladesh highlighted the role of religious leaders in raising awareness about NCDs. By training over 100 Imams and incorporating health topics like diet, physical activity, and sugar reduction into Friday sermons, the program

reached 20 million people. Female staff were involved in screening to increase women's participation, and 100 "Smart Schools" were established to promote healthy eating by avoiding junk food.

2. Nepal emphasized a multi-sectoral approach to tackle childhood obesity, a leading cause of Type 1 diabetes. Dr Pushpa Raj Poudel pointed out that country had implemented legislation to promote breastfeeding, introduced incentives, and launched programs like "My Health, My Responsibility." A kitchen and dining program in schools was also initiated to improve nutrition, alongside a comprehensive NCD plan that integrates mental health and NCD prevention.
3. Strategies for delivering healthy lifestyle interventions through mobile clinics and public health nursing officers in Sri Lanka were discussed by Ms N.D.C. Ruwanthi. She highlighted that in country NCD screening is conducted for individuals above 35 years, with education on physical activity, tobacco, and alcohol use provided to individuals and groups. Family-centered education during field visits ensures a holistic approach to diabetes prevention.
4. Dr Indah Suci Widyahening, representing academic and research institute focused on generating evidence to promote healthy settings, particularly in universities. Giving example of Indonesia, she highlighted country is transforming academic institutions into "healthy universities" by integrating health promotion into all programs. Universities are also involved in primary care transformation, with a focus on health promotion research and translating evidence into policy. A healthy university network has been launched as part of ASEAN initiatives, targeting primary interventions like health promotion for students.

   Following were **Key takeaways** from this session.

   - invest in programmes to promote healthy lifestyle interventions.
   - promote community-led initiatives grounded in local culture and traditional knowledge including to reduce the stigma.
   - adopt healthy setting-based approaches to educate and counsel families and people living with diabetes to make healthier choices and improving awareness about diabetes prevention and self-care management through targeted information and communication campaigns.
   - strengthen and enforce public policies, laws, regulations, and other measures to address the drivers of obesity and other risk factors for diabetes.
   - regulate marketing practices, especially those targeting children, to reduce the consumption of unhealthy foods containing excessive sugars, sodium, saturated fats and trans fats, creating a favourable food environment that promotes healthy lifestyle options.

Learnings from the field visits

Field visits were arranged by Ministry of Health Sri Lanka at three locations (Primary Medical Care Unit Kalagedihena; Primary Medical Care Unit Rukmalgamma; and District Hospital Athurugiriya). In the public health sector of Sri Lanka, curative services are provided through an extensive hierarchical network of institutions ranging from primary medical care units (PMCUs), divisional hospitals (DHs), base hospitals (BHs), district general hospitals (DGHs), provincial general hospitals (PGHs), special hospitals and teaching hospitals (THs). PMCUs and DHs deliver primary medical care. They are manned by MBBS qualified doctors without specialist qualifications. BHs constitute the secondary level of care. They provide specialized services in general surgery, general medicine, obstetrics and gynaecology and paediatrics, in addition to outpatient services. The DGHs, PGHs, THs, special hospitals and the National Hospital of Sri Lanka constitute the tertiary level of care and provide services in a wide range of specialties. These are managed by the MoH. All curative services provided in the government health system are free of charge for the patient at the point of delivery.

The field visits provided an opportunity of experiential learning of the screening, referral, treatment follow-up, and community engagement strategies employed in primary health care institutes in Sri Lanka to manage diabetes and other NCDs.

Objectives

- Showcase current practices for screening for risk factors, early diagnosis and treatment and referral from primary care

Interaction with field visit groups

- Showcase practices of awareness and community engagement with specific focus on prevention
- Observe how prevention and treatment are integrated within the system

Dr Farrukh Quershi from WCO Sri Lanka and Ms Sanjana Marpadga from WHO HQ moderated the session to collate reflections from the field. Interactive observations were shared by participating countries, focusing on both strengths of NCD care in Sri Lanka and potential areas for improvement. These discussions were informed by their own national experiences as well as insights gained from field visit observations in three main domain – prevention, treatment, and integration with other primary care needs.

Reflections from three groups is summarized as strengths of the NCD prevention and management services in Sri Lanka. The groups also provided recommendations to improve the service delivery in Sri Lanka as well as for other countries based on their maturity of health system.

Strengths

- Implementation of proactive outreach activities to raise health awareness at community and school levels, showcasing a strong connection between health facilities and the community. Emphasis on teamwork for healthy lifestyle interventions using a life-course approach.

Field visit at Primary Medical Care Unit Kalagedihena showcasing integration of NCD interventions at healthy lifestyle clinics with treatment centers

- Seamless integration of healthy lifestyle clinics with treatment centers to ensure efficient referral and follow-up systems.
- Effective management of diabetes and its complications, including screening, treatment, systematic recording, and reporting.
- Established mechanisms for community support activities at health facilities for health promotion, elderly care, mental health, and community reach-out activities through mobile clinics.

Recommended areas for improvement

- Need for robust digital facility-based monitoring system, and scale-up of protocol-based management at primary health care level.
- Ensuring noncommunicable disease services are aligned with the progress achieved in areas like infectious diseases and maternal and child health services.
- Greater engagement is needed of non-health sectors and government departments to further strengthen prevention of diabetes.

Formulation of Colombo Call to Action for strengthening prevention and control of diabetes in the WHO South-East Asia Region

Presenters:

Dr S C Wickramasinghe, DDG NCD, Ministry of Health, Sri Lanka

Dr Alaka Singh, WHO Representative to Sri Lanka

Reflection from Drafting Committee Members

- Dr Karchung, Deputy Chief Program Officer, Ministry of Health, Bhutan
- Mr Abdulla Muaz Adam, Public Health Coordinator, Ministry of Health, Maldives
- Dr Pushpa Raj Poudel, Senior Consultant, Ministry of Health and Population, Nepal

Process of formulation of the Colombo Call to Action

Drafting committee representing nominated officials from nine Member States of the region, along with technical experts attending the meeting were set-up to draft the Colombo Call to Action

The key takeaways from each technical session discussions (technical sessions A to D, poster presentation (Annex 2) was presented to drafting committee to deliberate and identify catalytic action points that are feasible, implementable in context with countries and region

Drafting committee additionally reviewed the Governing bodies resolutions, Global Diabetes Compact Key Asks, framework and strategies adopted by other WHO regions to frame the catalytic action.

The Call to Action prepared by drafting committee was shared with participants of the meeting to provide any further input, and relevant suggestion were incorporated in consultation with Drafting committee.

The revised version was presented by Dr S. C. Wickramasinghe from Sri Lanka and Dr Alaka Singh. Subsequently, **three members (Bhutan, Maldives and Nepal)** of the drafting committee were invited to share their reflections on the relevance of Call to Action

Reflections

Drafting committee Member from Bhutan shared that country recognized the critical importance of addressing burden of diabetes in the country and welcome Colombo Call to Action is timely and seeks to unite the government and stakeholders in implementing sustainable, evidence-based strategies. He pointed out that key focus areas of Call to Action include promoting healthy lifestyles, raising diabetes awareness, ensuring equitable healthcare access through setting-based approaches, strengthening public policies, and integrating traditional and modern care for informed, healthier choices. By building on the SEAHEARTS framework, Bhutan could set ambitious milestones for diabetes prevention and control by 2030, emphasizing measurable progress.

The process of developing Colombo Call to Action through a comprehensive engagement process and outlining practical and achievable action points tailored to the country's priorities was acknowledged by drafting committee member from Nepal. He reflected that the feasibility of catalytic actions outlined in Call to Action were carefully assessed based on available resources, country capacity, and health system challenges. Each action was reviewed in detail to ensure alignment with national capabilities and can be targeted to the fourth UN High-Level Meeting.

Drafting committee Member from Maldives indicated that given the high prevalence of undetected and untreated diabetes, Colombo Call to Action provides a crucial opportunity

Reflections from Drafting committee members on Colombo Call to Action

for the country to strengthen its response by integrating efforts across the entire care continuum. He highlighted that key strategies include developing subnational protocols, enhancing health management information systems for longitudinal tracking across public and private sectors, and promoting universal health coverage with an essential diabetes care package that ensures financial protection. He indicated that Call to Action is timely and necessary, as it enables diabetes care to serve as a tracer for strengthening all aspects of the health system.

At the end of session, countries were requested to submit additional inputs on the Colombo Call to Action draft within the next 2 weeks. It was also recommended to use Colombo Call to Action as the base for a Resolution on renewed SEAHEARTS milestones beyond 2025 at the Seventy-Eighth Session of the WHO Regional Committee Meeting of South-East Asia to be held in Sri Lanka in 2025. The final Call to Action is indicated at Page 28 of the report.

Leveraging partnerships for accelerating national response for prevention and management of diabetes

Session moderators	
Mr Kunle Adeneyi Country Representative Unfpa, Sri Lanka.	
Dr Susie Perera, Consultant, WHO Country Office, Sri Lanka.	
Panelists	Ms Beatriz Yanez Jiménez, International Diabetes Federation
	Ms Mette Skar, Senior Programme, Manager World Diabetes Foundation
	Ms Di Dong, World Bank Country Office, Sri Lanka
	Mr Christian Skoog , UNICEF
	Mr Austin Davis, NORAD (virtual)
	Ms Danielle Cazabon, Resolve to Save Lives (virtual)

Session objectives

- To understand initiatives undertaken by partners on key asks of GDC, and how countries benefited across regions
- To solicit commitments of the stakeholders for strengthening prevention and control of diabetes in the WHO South-East Asia Region (Colombo Call to Action)

The panel discussion on leveraging partnerships explored innovative approaches, resource mobilization, and policy integration to strengthen diabetes care

Key points highlighted by partners were:

1. **Education and empowerment for diabetes awareness:** Ms Beatriz Yanez Jiménez from International Diabetes Federation (IDF) emphasized the importance of education in diabetes prevention and care. The IDF School of Diabetes, with over 100 000 healthcare professionals (HCWs) enrolled globally, offers courses in 75 languages, enhancing

Bringing stakeholders together to bridge the gaps in diabetes prevention and management

knowledge beyond clinical aspects to improve doctor-patient communication. Additionally, the IDF Congress, held biennially, provides a platform for discussions on patient well-being. For policymakers, the IDF Diabetes Atlas compiles critical data on diabetes prevalence and financial burden, supporting evidence-based advocacy.

2. **Innovative, cost-effective solutions for diabetes care:** Importance of sustainable diabetes prevention and care models tailored to specific regional challenges were stressed by Ms Mette Skar from World Diabetes Foundation (WDF). With over 600 projects across 120 LMICs, WDF supports initiatives in prevention, early diagnosis, and treatment. Key strategies include strengthening health systems, developing treatment protocols, and integrating diabetes care into PHC. WDF also promotes mobile clinics, telemedicine, and school health programs to improve healthcare accessibility, particularly in rural areas.
3. **Strengthening primary health care systems:** The need for robust PHC systems to manage NCDs including diabetes was highlighted by Ms Di Dong from World Bank . With a $6 billion investment in 26 health projects across South Asia, the World Bank aims to improve healthcare infrastructure, provide essential medicines, and enhance data-driven decision-making. Performance-based financing is being implemented in countries like Sri Lanka and India, incentivizing improved diabetes follow-ups and patient outcomes.
4. **Life course approach to diabetes prevention:** Mr Christian Skoog from UNICEF underscored the significance of early interventions, addressing diabetes risk factors from pre-pregnancy to childhood and adolescence. Sri Lanka's strong maternal care system is being enhanced with updated maternal care packages, newborn nutrition

initiatives, and exclusive breastfeeding support. Additionally, UNICEF promotes school health programs, food policy regulations, and community-led health education to create a healthier future generation.

5. **Data-driven approaches for patient tracking:** The importance of offline mobile platform that allows healthcare workers to track hypertension and diabetes outcomes (Simple App) was pointed out by Ms Danielle Cazabon from Resolve to Save Lives (RTSL). This app, now in 11,000 public health facilities worldwide, has registered over 2.5 million hypertension patients and nearly 1 million diabetes patients, demonstrating improved patient management. Countries like Bangladesh, Sri Lanka, and Nigeria have integrated this tool into national health systems.
6. **Multi-sectoral collaboration and resource mobilization:** Mr Austin Davis from NORAD and other panelists stressed the importance of catalytic funding to improve health systems, advocating for affordable diabetes treatment, simplified care protocols, and greater investment in PHC. Success in diabetes care depends on partnerships with governments, private sectors, and civil society organizations.

Panelists also expressed strong support for aligning with the Colombo Call to Action (annex 3), emphasizing government accountability, resource allocation, and community engagement. They highlighted the need for systemic improvements, integrated policies, and a patient-centered approach to ensure sustainable, high-quality diabetes care.

Closing session

Deputy Minister of Health and Media, Sri Lanka interacting with participants

His Excellency, Dr Hansaka Wijemuni, Deputy Minister of Health and Media, Sri Lanka, attended the closing session and delivered remarks based on the event summary presented by Dr Hans V. Hogerzeil and discussions from the Colombo Call to Action session.

Dr Wijemuni emphasized that diabetes is a major public health issue, ranking as the third leading cause of death, with 50% of patients not receiving treatment. Sri Lanka is committed to being part of the Global Diabetes Compact (GDC) and achieving coverage targets, ensuring quality medications for all diabetes patients.

He highlighted the crucial role of social and media platforms in raising awareness, preventing risk factors, and promoting early screening.

Identified solutions, cross-learning, field experiences, and shared knowledge will guide future actions. The Colombo Call to Action provides a framework for coordinated efforts, and the government is fully committed to combating diabetes and other NCDs.

He mentioned that Sri Lanka will implement catalytic actions outlined in Call to Action, stressing the need for serious commitment from all stakeholders. The country has already initiated SEAHEARTS, ensuring universal access to diagnostics, treatment, and essential medicines. The focus is not just on policies and programs but on making a real impact on the millions living with diabetes. Increased engagement and networking are essential for achieving these goals.

The event was officially closed by Dr P. G. Mahipala, Secretary of the Ministry of Health, Sri Lanka.

Participants engaged in insightful discussions

Walk the Talk – Healthy break session

3 Meeting outcome

Colombo Call to Action: Strengthening prevention and control of diabetes in the WHO South-East Asia Region

Regional Commemoration of World Diabetes Day 2024
'Breaking barriers, Bridging gaps'

21–22 November 2024
Colombo, Sri Lanka

World Health Organization
REGIONAL OFFICE FOR South-East Asia

Colombo Call to Action: Strengthening prevention and control of diabetes in the WHO South-East Asia Region

We, the participants of the World Diabetes Day Regional Commemoration at WHO South-East Asia Region held in Colombo, Sri Lanka, on 21–22 November 2024,

1. EXPRESSING DEEP CONCERN about the growing burden of diabetes in the WHO South-East Asia Region, with approximately 246 million people aged 30 years and above living with diabetes and approximately another 500 000 people dying of diabetes annually therein, with more than half of cases occurring prematurely among those aged less than 70 years;

2. RECOGNIZING that only one in three adults with diabetes in the Region receives treatment, and more than 90% of people with untreated diabetes remain undiagnosed;

3. NOTING that more than 260 000 children and adolescents live with type 1 diabetes, with the most among them having limited access to insulin, insulin delivery devices and blood glucose self-monitoring devices that are required for optimum control of the disease, alongside a rising incidence of type 2 diabetes among young people;

4. COGNIZANT OF THE FACT that the burden of diabetes can be significantly reduced through efforts focused on the prevention and control of key risk factors, including obesity, unhealthy diet, physical inactivity and tobacco use;

5. ACKNOWLEDGING that strengthening primary health care is the foundation for accessible NCD services for universal health coverage, with diabetes prevention and management interventions integrated within broader NCD programmes, maternal and child health programmes, and relevant infectious disease programmes;

6. RECALLING

   (i) World Health Assembly resolutions and Decisions:

      (a) Resolution WHA74.4: Reducing the burden of noncommunicable diseases through strengthening prevention and control of diabetes;

      (b) Decision WHA75.11: Follow-up of the Political Declaration of the Third High-Level Meeting of the UN General Assembly on the prevention and control of noncommunicable diseases in May 2022, which adopted five global diabetes targets to be achieved by 2030;

      (c) Resolution WHA72.8: Improving the transparency on markets for medicines, vaccines and other health products;

   (ii) Regional Committee for South-East Asia resolutions:

      (a) Implementation Roadmap for the prevention and control of noncommunicable diseases (NCDs) in South-East Asia 2022–2030 adopted through Regional Committee resolution SEA/RC75/R2 focuses on monitoring progress and the accelerating interventions for NCDs, including oral health and integrated eye care. It emphasizes prioritizing the most impactful actions to achieve the five global coverage targets for diabetes agreed by Member States in Decision WHA75.11,

      (b) SEAHEARTS: Accelerating prevention and control of cardiovascular diseases in the South-East Asia Region (resolution SEA/RC76/R5) to reach time-bound targets and milestones;

   (iii) The WHO Global Diabetes Compact with the vision to reduce the risks of diabetes and ensure that all people who are diagnosed with diabetes have access to equitable, comprehensive, affordable, and quality treatment and care;

7. RECOGNIZING that diabetes should be covered through the life course and that optimum diabetes management requires access to affordable, quality-assured insulin and oral glucose-lowering medication, associated health technologies, diagnostic and monitoring capacity, education and counselling, and support from health-care providers and patient groups;

8. RECOGNIZING that equitable access to quality, affordable essential medicines and health technologies, including those for diabetes, is part of the progressive realization of the fundamental right to the highest possible standard of health (in short: "the right to health");

9. NOTING that significant progress has been made towards achieving the target of 100 million people with diabetes and hypertension to be placed on protocol-based management by the end of 2025;

10. REITERATING support to designing a new, ambitious and achievable Political Declaration at the Fourth UN High-Level Meeting on NCDs in September 2025;

Call upon national governments and nongovernmental organizations to:

11. STRENGTHEN efforts to implement SEAHEARTS interventions to further improve the treatment cascade of hypertension and diabetes in primary health, with the aim to achieve the SEAHEARTS milestones fully by 2025;

12. ALIGN national efforts with technical guidance provided by the WHO Global Diabetes Compact;

13. ADVOCATE for regional agreement on renewed SEAHEARTS milestones beyond 2025, leveraging the present momentum to accelerate the prevention and control of diabetes, hypertension and thereby, cardiovascular diseases in general;

14. ENSURE that universal health coverage is promoted so that all people living with diabetes have access to an essential package of care and treatment without out-of-pocket expenditures or financial hardship at the point of care;

15. INVEST in programmes to promote healthy lifestyle interventions by:

    (i) promoting community-led initiatives grounded in local culture and traditional knowledge including efforts to reduce stigma for people living with diabetes;

    (ii) adopting healthy setting-based approaches to educate and counsel families and people living with diabetes to make healthier choices and improving awareness about diabetes prevention and self-care management through targeted information and communication campaigns;

    (iii) strengthening and enforcing public policies, laws, regulations and other measures to address the drivers of obesity and other risk factors for diabetes;

    (iv) regulating marketing practices, especially those targeting children, to reduce the consumption of unhealthy foods containing excessive sugars, sodium, saturated fats and transfats, and creating a favourable food environment that promotes healthy lifestyle options;

16. INTEGRATE diabetes prevention and management into the existing programmes at the primary health care level as steps towards the achievement of universal health coverage, by:

    (i) identifying opportunities to ensure that diabetes prevention and management interventions are fully integrated into existing programmes at the primary health care level for other NCDs (e.g. hypertension), maternal and child health care, and tuberculosis and HIV services;

    (ii) integrating an essential package of diabetes diagnosis, treatment and care into universal health coverage benefit packages to ensure the delivery of services that are promotive, preventive, rehabilitative and palliative, as well as focused on effective management throughout the life course;

17. STRENGTHEN the capacity of health systems to:

    (i) offer testing to identify individuals with diabetes, ensuring linkage to the care cascade across all levels of care, including diagnosis, treatment and management, and in addressing complications;

    (ii) achieve better glycaemic control for people living with diabetes receiving care, through full implementation of nationally or subnationally approved treatment protocols and approaches of WHO HEARTS-D technical packages;

(iii) provide healthy lifestyle counselling;

(iv) implement health management information systems, preferably electronic, to enable longitudinal tracking of people living with diabetes in both public and private sectors. These systems should generate information on national and global targets and serve as a management tool to improve care at the facility level;

18. INVEST in interventions to improve access to diabetes diagnostics, medicines and health products, particularly insulins, and their scientifically sound and cost-effective use by:

(i) strengthening the capacity of national regulatory authorities to ensure all diabetes products on the market are quality-assured (including the evaluation of biosimilar insulins);

(ii) adapting robust forecasting tools for realistic quantification of requirements as per treatment protocol (including up-to-date patient registries);

(iii) ensuring adequate financing to meet requirements through free supply or social health insurance;

(iv) strengthening national procurement mechanisms by incorporating market assessments, supplier prequalification, centralized issuance of tenders and other best practices for goods procurement and facilitate sharing of procurement prices regionally or globally;

(v) strengthening distribution systems, including the use of digitalized systems and monitoring stock-outs;

(vi) scaling up the cold chain infrastructure to ensure the quality of insulin;

(vii) regulating prices and distribution mark-ups and eliminating taxes on essential diabetes medicines and blood glucose monitoring devices;

(viii) monitoring the availability and prices in facilities;

(ix) prioritizing access to HbA1c testing at primary health care facilities in addition to testing blood glucose using a meter and test strips;

(x) educating health-care providers through clinical guidelines (including how to switch between insulin types and brands), and raising awareness on diabetes prevention and treatment among people living with diabetes and the public in general;

19. INVEST in implementation research to foster evidence-informed diabetes prevention and treatment programme design and implementation; and

20. ESTABLISH national targets aligned to the global diabetes coverage targets to be achieved by 2030, promoting accountability through timely, reliable and sustained national data on risk factors, diseases and mortality, and

Call upon international partners, development agencies, global health organizations, academia and WHO collaborating centres to:

21. SUPPORT the implementation of actions by countries and the WHO Secretariat; and

Request the WHO Secretariat to:

22. PROVIDE technical support to countries in designing national roadmaps to reach the global diabetes targets within the roadmaps for renewed SEAHEARTS milestones beyond 2025 and aligned with technical guidance provided by the WHO Global Diabetes Compact;

23. SUPPORT forums for political and technical discussions; and

24. PROMOTE research into innovative solutions and documentation and dissemination of best practices.

Annexure 1

Regional Director's speech on Regional Commemoration of World Diabetes Day 2024 'Breaking barriers, Bridging gaps' 21–22 November 2024, Colombo, Sri Lanka

Excellencies and officials from Government of Sri Lanka

Officials and dignitaries from our Member States

Experts, Partners, Colleagues and friends

Good morning, and welcome to this Regional Commemoration of World Diabetes Day.

Before I go any further, my thanks to the Government of Sri Lanka for organising this event and bringing much-needed attention to the problem of diabetes.

Our theme today, "Breaking Barriers, Bridging Gaps," reminds us of the barriers - behavioural, socio-economic and within health systems - to addressing diabetes.

It calls on us to collectively act on evidence-based solutions and innovations to overcome these barriers, while accelerating cost-effective interventions for diabetes prevention, diagnosis, and management.

In our region, one in every ten adults is living with diabetes - yet only half of them are aware of their condition. Worryingly, less than one in four adults is receiving treatment.

Given this backdrop, I would like to acknowledge and commend the Government of Sri Lanka for its efforts to bring diabetes to the forefront as a public health issue. The outcome of this event, the Colombo Call to Action, will be an important step towards diabetes prevention and management in the region.

Sri Lanka has made significant progress in addressing risk factors for non-communicable diseases, such as unhealthy diets and physical inactivity. Notably, Sri Lanka is implementing new regulations to eliminate trans fats, and also establishing a network of healthy lifestyle centres to expand health promotion, prevention, and NCD screening facilities.

Countries and partners must take action to meet the Global Diabetes Compact targets by 2030. We are at a history-defining juncture, and I urge you all to use this event to decisively commit to the work ahead.

I am proud that our South-East Asia countries have committed to four ambitious SEAHEARTS milestones by 2025, including ensuring that 100 million people with diabetes and hypertension are on protocol-based management. I am pleased to say that as of June 2024, our countries have already placed over 59 million people on such protocol-based management.

However, we still have a long way to go.

Our primary healthcare facilities must be equipped to meet the increasing demand for preventive and management services. This includes the use of standard treatment protocols across all facilities, the availability of essential medicines, affordable and quality diagnostics, and well-trained healthcare professionals.

Your deliberations over the next two days will be focused on the core themes of TREAT, INTEGRATE, PREVENT, and UNITE. I hope they will result in actionable recommendations to achieve key targets we all want to achieve, including reducing diabetes risk factors.

I urge everyone to make the most of this opportunity to learn, share experiences, and collaborate.

To all the government representatives present, and to all our partners, I will end by restating my commitment to supporting all your efforts. WHO stands beside you, our Regional Office stands beside you, and I stand beside you.

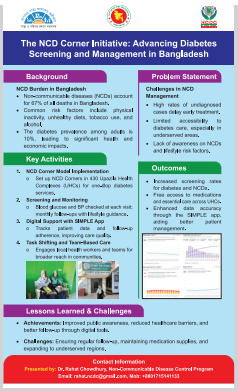
Thank you.

Annexure 2

Posters – Good practices by countries to prevent and manage diabetes

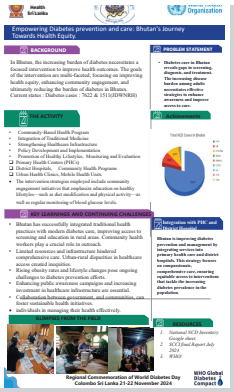
The session provided an opportunity of cross learning of good practices by countries on prevention and management of diabetes. The poster presented are detailed below:

Community mobilization and awareness efforts through religious leaders to prevent and manage diabetes in Bangladesh



The NCD corner initiative: advancing diabetes screening and managment in Bangladesh

BANGLADESH

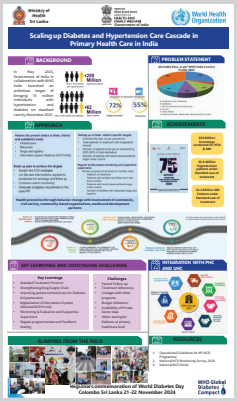


Integration of traditional health practice with modern care to improve access to screening and education for diabetes in Bhutan's journey towards health equity

BHUTAN

Community outreaches and screening services at work and public places, integrated home visits- success of Ghana's AKOMA PA programme to enroll 75,000 people with diabetes

GHANA



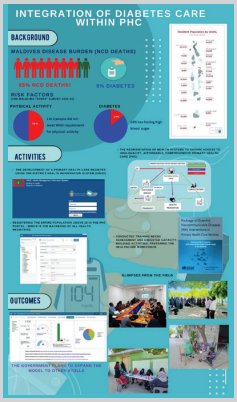
Political commitment and using foundation of primary health care- India's progress story to put 21 million people with diabetes on standard care

INDIA

Improving diabetes detection at community level through ensuring availability of diabetes technologies – Indonesia's 14 priority disease screening intervention

Integration of TB-DM management in primary health care services and improvement of treatment monitoring through HbA1c testing

INDONESIA



Integration of diabetes care within primary health care and use of digitalized longitudinal monitoring to track progress in Maldives

MALDIVES

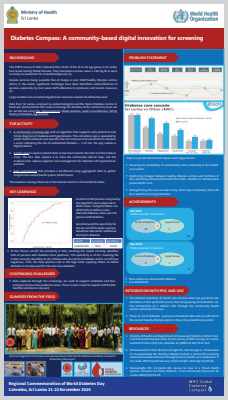
24 weeks structured lifestyle medicine programme for newly diagnosed diabetes in primary health care to achieve diabetes remission in Malaysia

Monitoring glycemic control and quality of care of diabetes - National Diabetes Registry in Malaysia

MALAYSIA

Integration of diabetes services in PHC system – walking the path towards universal health coverage in Nepal

NEPAL



Implementation research for use of digital technologies for screening at community level through community screening app and clinic -Diabetes Compass in Sri Lanka

Improving diabetes treatment outcomes through protocol-based management of diabetes in Sri Lanka

SRI LANKA

Strong public health policies, Community Health Station programme, engagement of local authorities- a positive step towards prevention of diabetes in Thailand

Integration of diabetes remission in NCD clinic with quality assessment to achieve the good glycaemia control in Thailand

THAILAND

Use of drug forecasting tools and simplified management protocols at primary health care in Timor Leste

Introduction of Pro health taxes on Sugar Sweetened Beverages, tobacco and alcohol to reduce diabetes risk factors in Timor Leste

TIMOR-LESTE

Annexure 3

Programme

Time (IST)	Session	Facilitator/ speaker
Day 1: 21 November 2024		
08:00–09:00	Registration	
09:00–09:40	Inaugural Session • Welcome remarks: Dr S C Wickramasinghe, Deputy Director General (NCD), Ministry of Health, Sri Lanka • Message: Dr Tedros Adhanom Ghebreyesus, Director-General, World Health Organization (video) • Message: Ms Saima Wazed, WHO Regional Director for South-East Asia (video) • Address: Dr Guy Fones, Director (NCD) a.i., NCD Department, WHO Headquarter • Voice of a person living with diabetes (video) • Address: Dr P G Mahipala, Secretary, Ministry of Health, Sri Lanka • Digital launch of SEAHEARTS Monitoring Platform • Vote of Thanks: Dr Samiddhi Samarakoon, Director (NCD), Ministry of Health, Sri Lanka	
09:40–10:45	Group Photo and Healthy Break E-poster interactive walk-through: to view and interact with the presenters of the e-posters showcasing good practices by countries on the prevention and management of diabetes	
	Technical session A: Key priorities of the Global Diabetes Compact Moderators: • Dr S C Wickramasinghe, Deputy Director General (NCD), Ministry of Health, Sri Lanka • Dr Guy Fones, Director (NCD) a.i., NCD Department, WHO Headquarter	
10:45–10:55	Essentials of the WHO Global Diabetes Compact: what it means at regional level(virtual)	Dr Bianca Hemmingsen, Medical Officer (Diabetes Control), WHO HQ
10:55–11:10	Inspiring Success: Global lessons from the key asks of WHO Global Diabetes Compact	Dr Nikhil Tandon, Chair, WHO Global Technical Advisory Group on Diabetes

Time (IST)	Session	Facilitator/ speaker
11:10–11:20	Synthesis of country efforts and an overview of the SEAHEARTS initiative to accelerate the prevention and control of diabetes in WHO South-East Asia Region	Dr Pradeep Joshi, Technical Officer (NCD), WHO South-East Asia Regional Office
11:20–11:30	Question and Answer	
	Technical session B: Bridging access: closing the gap for diabetes essential medicines, technologies, and health products Moderators: • Mr Nayaz Ahmed, Deputy Director General, Ministry of Health, Maldives • Dr Nalika Gunawardena, Regional Adviser (NCD), WHO South-East Asia Regional Office	
11:35–11:50	Barriers to available, affordable and quality-assured insulin and key pathways to access	Dr Margaret Ewen, Senior Projects Manager, Health Action International
11:50–12:05	Bringing diagnostic closer: improving access to diabetes technologies in primary health care	Dr Beatrice Vetter, Director NCDs, FIND
12:05–12:35	Moderated discussion with panelists on good practices and solutions based on the country level experience to improve availability and access to diabetes medicines, and technologies Panelists: • Dr Esti Widiastuti Mangunadikususmo, Senior Health Epidemiologist, Ministry of Health, Indonesia • Dr Anupam Prakash, Head of Medicine Department, Lady Hardinge Medical College, India • Dr Helder Juvinal Neto da Silva, PEN Unit Officer, Ministry of Health, Timor-Leste • Ms Kim Uhjin, Regional Adviser (Essential drugs and medicines), WHO South-East Asia Regional Office	
12:35–12:45	Question and Answer Summarizing catalytic actions by the countries from the session for the Colombo Call to Action	
13:30–13:50	Technical session C: Universal diabetes care: Integrating care and tracking progress Moderators: • Mr Saroj Kumar, Director, Ministry of Health and Family Welfare, India • Dr Gampo Dorji, Team Lead (NCD and Health systems), WHO Nepal	
	Integrating diabetes prevention and management at primary health care and universal health coverage: examples from low- and middle-income countries	Dr Hans V Hogerzeil, Professor, Faculty of Medical Science, University of Groningen

Time (IST)	Session	Facilitator/ speaker
13:50–14:45	Moderated discussion with panelists on country level experience in integrating diabetes prevention and management in PHC for UHC, and in tracking coverage targets for diabetes care Panelists: • Dr Churit Tengtrisorn, Medical Officer, Ministry of Public Health, Thailand • Dr Karchung, Deputy Chief Program Officer, Ministry of Health, Bhutan • Mr Abdulla Muaz Adam, Public Health Coordinator, Ministry of Health, Maldives • Dr James Duah, Deputy Executive Director, Christian Health Association of Ghana • Dr Bishwajit Bhowmik, Director, Executive Diabetes Care Centre,BADAS, Bangladesh	
14:45–15:00	Question and Answer Summarizing catalytic actions by the countries from the session for the Colombo Call to Action	
15:00–15:30	Technical session D: Healthier communities, healthier living, healthier future: preventing diabetes and promoting wellbeing Moderators: • Dr Sivarajan Ramasamy, Senior Principal Assistant Dir, Ministry of Health, Malaysia • Dr Bhupinder Kaur Aulakh, WHO Representative to Bhutan	
15:35–15:50	From prevention to remission: tackling diabetes with effective strategies and solutions	Dr Weerachai Srivanichakorn, Associate Professor, Faculty of Medicine Siriraj Hospital Mahidol University, Thailand
15:50–16:05	Balancing blood sugar: the power of dietary interventions	Dr Khadar Vali Dudekula, Independent Scientist, Vishwa Manavata Samastha, India
16:05–16:35	Moderated discussion with panelists on good practices and innovative solutions based on country level experience preventing diabetes and promoting wellbeing of those living with diabetes Panelists: • Dr Rahat Iqbal Chowdhury, Deputy Program Manager, Ministry of Health and Family Welfare, Bangladesh • Dr Pushpa Raj Poudel, Senior Consultant, Ministry of Health and Population, Nepal • Ms. N.D.C. Ruwanthi, Public Health Nursing Officer, Ministry of Health, Sri Lanka • Dr Indah Suci Widyahening, Professor, Faculty of Medicine Universitas Indonesia	

Time (IST)	Session	Facilitator/ speaker
16:35–16:45	Question and Answer Summarizing catalytic actions by the countries from the session for the Colombo Call to Action	
16:45–17:00	Briefing for the Field Visit	
Day 2: 22 November 2024		
08:30–09:00	Travel for field visit	
09:00–12:30	Field visits to: • showcase current practices for screening for risk factors, early diagnosis and treatment and referral from primary care • showcase practices of awareness and community engagement with specific focus on Prevention • observe how prevention and treatment are integrated within the system	Sites: • Primary Medical Care Unit Kalagedihena, • Primary Medical Care Unit Rukmalgamma, • Base hospital, Homagama, • District Hospital Athurugiriya Coordinators: • Dr Amila Herath, Medical officer (NCD, Ministry of Health, Sri Lanka • Dr Susie Perera, Consultant, WHO Country Office, Sri Lanka
14:00–14:30	Observations from the field visits • Key observations and take aways • Experience of own countries Reflections • Dr Aizuniza binti Abdullah, Senior Principal Assistant Director Ministry of Health, Malaysia • Mr Sagar Prasad Ghimire, Ministry of Health and Population, Nepal	Facilitators • Dr Farrukh Qureshi, Team Lead (NCD), WHO Country Office, Sri Lanka • Dr Sanjana Marpadga, Technical officer (Diabetes), WHO HQ

Time (IST)	Session	Facilitator/ speaker
14:30–15:45	Panel discussion: Leveraging partnerships for accelerating national response for prevention and management of diabetes Moderators: • Mr Kunle Adeniyi, Representative for UNFPA Sri Lanka and Country Director for The Maldives • Dr Susie Perera, Consultant, WHO Country Office, Sri Lanka	
	Panelists: • Ms Beatriz Yanez Jiménez, Advocacy manager, International Diabetes Federation • Ms Mette Skar, Senior Programme Manager, World Diabetes Foundation • Dr Di Dong, Health Economist, World Bank Country Office, Sri Lanka • Mr Christian Skoog, Country Representative UNICEF, Sri Lanka • Mr Austin Davis, Senior Adviser, NORAD (virtual) • Ms Danielle Cazabon, Senior Program Manager Resolve to Save Lives (virtual) • Dr Gina Agiostratidou, Program Director, Helmsley Charitable Trust (virtual)	
16:15–16:45	Colombo Call to Action for strengthening prevention and control of diabetes in the WHO South-East Asia Region Presenters: • Dr S C Wickramasinghe, DDG NCD, Ministry of Health, Sri Lanka • Dr Alaka Singh, WHO Representative to Sri Lanka	
	Reflection from Drafting Committee Members • Dr Karchung, Deputy Chief Program Officer, Ministry of Health, Bhutan • Mr Abdulla Muaz Adam, Public Health Coordinator, Ministry of Health, Maldives • Dr Pushpa Raj Poudel, Senior Consultant, Ministry of Health and Population, Nepal	
16:45–17:00	Closing Session • Expert Observation: Dr Hans V Hogerzeil, Professor, Faculty of Medical Science, University of Groningen • Address: Hon. Dr Hansaka Wijemuni, Deputy Minister of Health and Media, Ministry of Health, Sri Lanka • Conclusion of the event	

Annexure 4

List of participants

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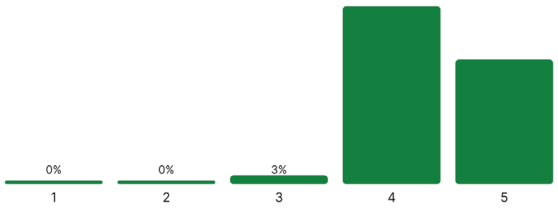
Annexure 5

Participant's feedback

1. To what extent the objectives of the meeting were accomplished?

Rating Poll 30 votes 30 participants

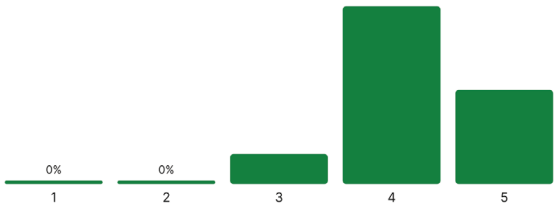
Score: **4.4**



2. The extent to which the agenda items were relevant to achieve objectives.

Rating Poll 29 votes 29 participants

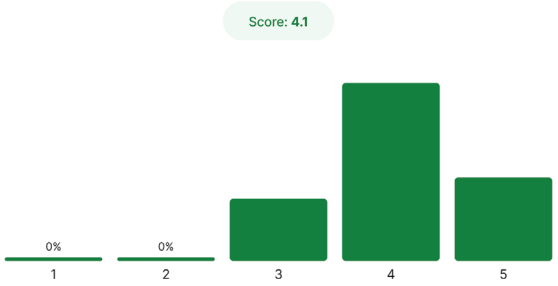
Score: **4.2**



3. The presentations and discussions were effective in getting to know the Key Asks of GDC and technical guidance

Rating Poll 31 votes 31 participants

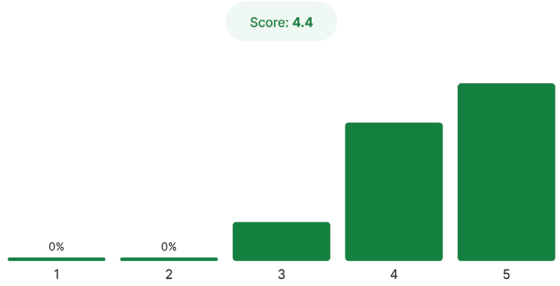
Score: **4.1**



4. Field trip: Was it relevant to achieve the objectives of the meeting?

Rating Poll 38 votes 38 participants

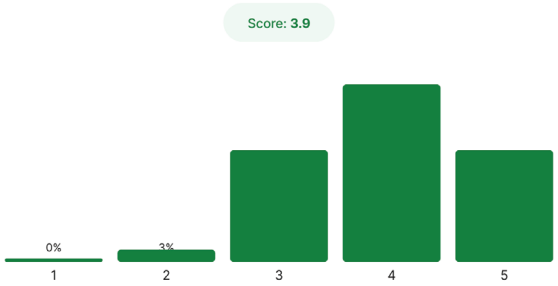
Score: **4.4**



5. The panel discussions were effective in clarifying the areas of coordinated support available for countries.

Rating Poll 37 votes 37 participants

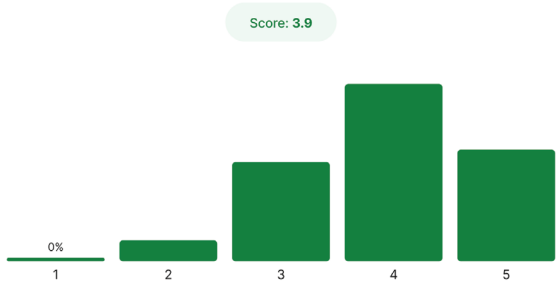
Score: **3.9**



6. The working papers presented were substantive to the needs of the meeting.

Rating Poll 37 votes 37 participants

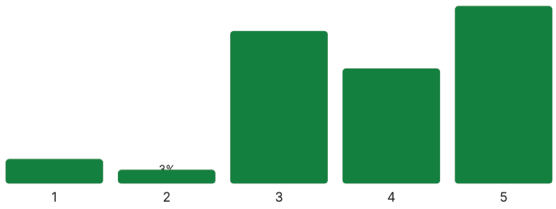
Score: **3.9**



7. Adequate opportunities were given to exchange information with other participants.

Rating Poll 38 votes 38 participants

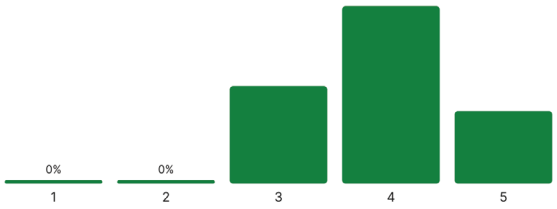
Score: **3.8**



8. The meeting was effective in getting to know how to accelerate actions to achieve the global diabetes targets.

Rating Poll 39 votes 39 participants

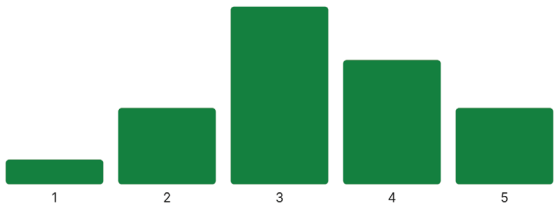
Score: **3.9**



9. Are you in a position to integrate the outcome of this meeting to the national work plan?

Rating Poll 38 votes 38 participants

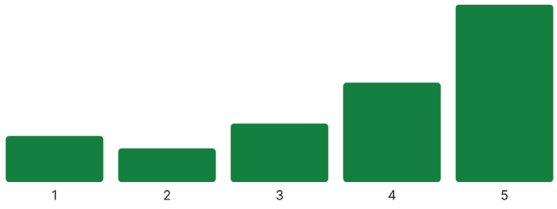
Score: **3.3**



Meeting arrangement feedbacks - meeting venue and accommodation

Rating Poll 37 votes 37 participants

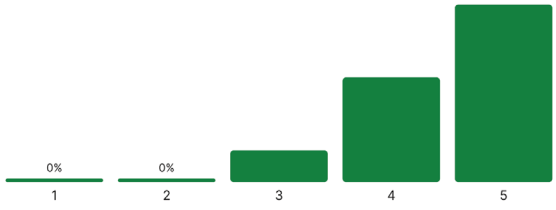
Score: **3.8**



Overall organization during the meeting

Rating Poll 39 votes 39 participants

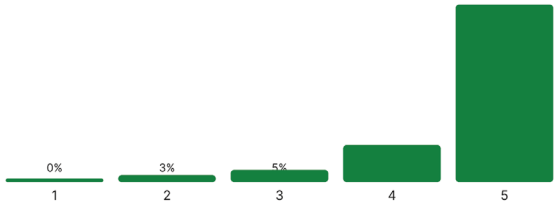
Score: **4.5**



Secretarial support -before and during meeting

Rating Poll 38 votes 38 participants

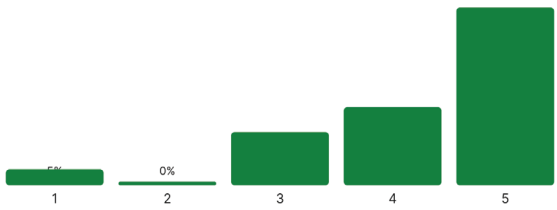
Score: **4.7**



Meals and healthy breaks including physical activity session

Rating Poll 37 votes 37 participants

Score: **4.2**



Annexure 6

Resource materials

1. Guide for participants, Regional Commemoration of World Diabetes Day: https://cdn.who.int/media/docs/default-source/diabetes/guide-for-participants.pdf?sfvrsn=fb4bc4bf_2
2. Colombo Call to Action strengthening prevention and control of diabetes in WHO South-East Asia Region https://cdn.who.int/media/docs/default-source/diabetes/colombo-declaration-call-to-action.pdf?sfvrsn=62d54183_3
3. WHO Diabetes Estimates (2022)- Worldwide trends in diabetes prevalence and treatment from 1990 to 2022: a pooled analysis of 1108 population-representative studies with 141 million participants - The Lancet
4. WHO Global Diabetes Compact https://www.who.int/initiatives/the-who-global-diabetes-compact/
5. Guidance on global monitoring for diabetes prevention and control: Framework, indicators and application https://iris.who.int/bitstream/handle/10665/379529/9789240102248-eng.pdf?sequence=1
6. Noncommunicable disease facility-based monitoring guidance https://www.who.int/publications/i/item/9789240057067
7. WHO HEARTS D Technical Package: https://www.who.int/publications/i/item/who-ucn-ncd-20.1
8. Prevention and management of diabetic foot problems WHO SEARO PEN-HEARTS OpenWHO courses to scale up capacity for NCD services in PHC
9. WHO South-East Asia Regional NCD Roadmap: https://apps.searo.who.int/whoroad/homev2
10. SEARO NCD dashboard: https://whosearo.viewzenlabs.in/home
11. Noncommunicable Diseases Data Portal: https://ncdportal.org/
12. The Global Health Observatory: https://www.who.int/data/gho
13. World Health Statistics: https://www.who.int/data/stories/world-health-statistics-2023-a-visual-summary/
14. Supporting member states in reaching informed decision-making on engaging with private sector entities for the prevention and control of noncommunicable diseases: a practical tool https://www.who.int/publications/i/item/9789240094840
15. WHO framework for meaningful engagement of people living with noncommunicable diseases, and mental health and neurological conditions https://www.who.int/publications/i/item/9789240073074

Ministry
of Health
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World Health
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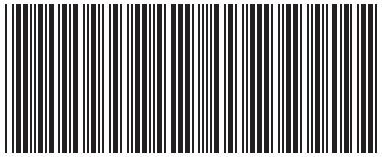

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